Kestrel Authorization Systems

KestrelPA — autonomous prior authorization for specialty clinics

Seed round | Austin, TX | kestrelauth.example

Confidential — for discussion purposes only

The Problem

Prior authorization consumes 13.7 hours per physician per week (AMA 2024 survey)

94% of physicians report care delays caused by PA; 24% report serious adverse events

Average specialty clinic employs 1 FTE per 3 providers purely for authorization work

Denials on first submission run 18-22% industry-wide, driving costly rework

Why Now

CMS Interoperability and Prior Authorization Final Rule (CMS-0057-F) mandates electronic PA APIs for impacted payers by January 2027

Payer FHIR endpoints becoming available creates a machine-readable submission path

Staffing shortages make the manual approach structurally unsustainable

Our Solution

KestrelPA reads the chart, assembles the clinical packet, and submits to the payer

Payer-specific rules engine covering 240 plans across ortho, GI and cardiology

Submits via FHIR where available, falls back to portal automation elsewhere

Clinic staff review an exception queue instead of doing data entry

Product Traction

8 paying specialty clinics; 3 in ortho, 3 GI, 2 cardiology

142,000 authorizations processed since January 2025

First-pass approval rate of 91% vs 79% clinic baseline (measured across 4 pilots)

Median turnaround 4.2 hours vs 3.1 days pre-deployment

Net revenue retention 118% over trailing 6 months

Business Model

SaaS subscription per provider per month, plus per-authorization usage fee

Land with a single specialty line, expand across the practice

Current ARR $840K; average contract value $105K

Gross margin 71% and improving as portal automation gives way to FHIR

Technology & Defensibility

Proprietary payer rules corpus: 240 plans, updated from 142K live submissions

US patent application filed (17/xxx,xxx) covering adaptive packet assembly

Every denial is labelled training data — the corpus compounds with volume

SOC 2 Type II complete; HIPAA BAAs in place with all customers

Competitive Landscape

Legacy clearinghouses (Availity, Waystar) — transaction plumbing, not clinical workflow

Point solutions focused on payer side rather than the specialty clinic

KestrelPA is 10x faster than the manual process and far easier to adopt

We are the only solution purpose-built for the specialty clinic exception queue

Go To Market

Direct sales to practice administrators; 62-day average sales cycle

Two channel conversations underway with specialty-focused EHR vendors

Specialty society conferences as primary top-of-funnel

Team

CEO — 11 years running revenue cycle operations for a 40-provider ortho group

CTO — former staff engineer, healthcare interoperability platform

VP Clinical Ops — RN, certified professional coder

9 FTE total; hiring two payer integration engineers

The Ask

Raising $6M seed to reach 40 clinics and $4M ARR

Use of proceeds: 45% engineering, 30% go-to-market, 25% payer integrations

18 months of runway at planned burn

Round is open; lead not yet committed

Unit Economics

Metric	Current	Target at $4M ARR
Average contract value	$105,000	$100,000
Gross margin	71%	78%
CAC payback	14 months	9 months
Net revenue retention	118%	125%